TALKING TO PAIN · EXTENDED TOOLKIT

The Clinic-Ready Companion Pack

Screeners, scripts, and patient worksheets to put the MI + CBT hybrid toolkit to work Monday morning. Print what you need; copy freely for use in your own practice.

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STarT Back (low back pain) — stratify, then match the care

What it is: a 9-item patient questionnaire from Keele University that sorts LBP patients into low / medium / high risk of poor outcome. Patients finish it in the waiting room. **Get the official form free** (clinical use) from Keele's STarT Back site — print a stack for the front desk.

Scoring	How
Items 1–8	Agree = 1, Disagree = 0
Item 9 (bothersomeness)	“Very much” or “Extremely” = 1; otherwise 0
Overall score	Sum of all 9 (0–9)
Psychosocial subscale	Items 5–9 only: fear of activity, anxious worry, catastrophizing, low mood/enjoyment, bothersomeness (0–5)

Risk tier	Rule	What to do
Low	Overall 0–3	Standard care + reassurance and pain education; avoid over-treating
Medium	Overall ≥4, subscale 0–3	Add structure: pacing, graded activity, SMART goals, closer follow-up
High	Psychosocial subscale ≥4	**Full MI + CBT toolkit, front and center.** These are your 2-Question Triage patients; consider co-management

The PHQ cascade — mood and anxiety

PHQ-4 on every new-patient intake (page 3). Score each pair: anxiety = Q1+Q2, depression = Q3+Q4. **Either pair ≥3 = yellow flag** → give the full PHQ-9 (page 4) for depression; consider GAD-7 for anxiety. Totals: 0–2 none · 3–5 mild · 6–8 moderate · 9–12 severe distress.

PHQ-9 interpretation: 0–4 minimal · 5–9 mild · 10–14 moderate · 15–19 moderately severe · 20–27 severe. **10+ = have the referral conversation.** Re-screen to track change — it's also an outcome measure.

ITEM 9 PROTOCOL — DECIDE THIS BEFORE YOU NEED IT

Any answer above “not at all” on PHQ-9 item 9 (thoughts of being better off dead or of self-harm) gets a **same-day, private, direct conversation** — ask plainly about thoughts, plan, and means. Keep at your desk: the **988 Suicide & Crisis Lifeline (call/text 988, US)**, plus two local behavioral-health contacts for warm handoffs. Document the conversation and the plan. Asking does not plant the idea — it opens the door.

Suggested office workflow: PHQ-4 inside the new-patient packet · STarT Back for every LBP patient at visit one and again at re-exam · scores recorded where you'll see them at the table · high psychosocial load ≠ “refer out” — it means the conversation IS the treatment, and MI + CBT move to the front.

PHQ-4 — how you've been feeling

PATIENT FORM

Name: _____ Date: _____

Over the last 2 weeks, how often have you been bothered by the following problems? Check one box per row.

	Not at all (0)	Several days (1)	More than half the days (2)	Nearly every day (3)
1. Feeling nervous, anxious, or on edge	☐	☐	☐	☐
2. Not being able to stop or control worrying	☐	☐	☐	☐
3. Feeling down, depressed, or hopeless	☐	☐	☐	☐
4. Little interest or pleasure in doing things	☐	☐	☐	☐

OFFICE USE

Anxiety (Q1 + Q2): _____ / 6 Depression (Q3 + Q4): _____ / 6 Total: _____ / 12

Either subscale ≥3 = positive screen → follow up (PHQ-9 for depression; consider GAD-7 for anxiety). Total: 0–2 none · 3–5 mild · 6–8 moderate · 9–12 severe.

The PHQ-4 (Kroenke, Spitzer, Williams & Löwe) combines the PHQ-2 and GAD-2. The PHQ family of instruments was developed with an educational grant from Pfizer Inc.; no permission is required to reproduce, translate, display, or distribute.

PHQ-9 — patient health questionnaire

PATIENT FORM

Name: _____ Date: _____

Over the last 2 weeks, how often have you been bothered by any of the following problems?

	Not at all (0)	Several days (1)	More than half the days (2)	Nearly every day (3)
1. Little interest or pleasure in doing things	☐	☐	☐	☐
2. Feeling down, depressed, or hopeless	☐	☐	☐	☐
3. Trouble falling or staying asleep, or sleeping too much	☐	☐	☐	☐
4. Feeling tired or having little energy	☐	☐	☐	☐
5. Poor appetite or overeating	☐	☐	☐	☐
6. Feeling bad about yourself — or that you are a failure or have let yourself or your family down	☐	☐	☐	☐
7. Trouble concentrating on things, such as reading the newspaper or watching television	☐	☐	☐	☐
8. Moving or speaking so slowly that other people could have noticed? Or the opposite — being so fidgety or restless that you have been moving around a lot more than usual	☐	☐	☐	☐
9. Thoughts that you would be better off dead or of hurting yourself in some way	☐	☐	☐	☐

If you checked off *any* problems: how difficult have these made it to do your work, take care of things at home, or get along with other people? ☐ Not difficult at all ☐ Somewhat ☐ Very ☐ Extremely

OFFICE USE

Total _____ / 27 · 0–4 minimal · 5–9 mild · **10–14 moderate** · 15–19 mod-severe · 20–27 severe · **10+ → referral conversation.**
Any item-9 answer above "not at all" → same-day protocol (p. 2).

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My SMART goal

PATIENT WORKSHEET

Pain shrinks lives. This worksheet grows yours back — one specific, checkable step at a time. Your clinician helps shape it; **you** choose it.

What I want back (the big picture — golf, grandkids, work, sleep…):

My goal for the next 1–2 weeks:

Make it…	Ask yourself	My answer
Specific	What exactly? Where? With whom?	
Measurable	What will I count — minutes, days, laps?	
Achievable	Could I do this even on a bad day? (If not — shrink it. A streak beats a distance.)	
Relevant	How does this connect to what I want back?	
Time-bound	When do I start? When do we review it?	

Start date: _____ We review it on: _____

How confident am I that I can do this? (circle one — below 7? Make the goal smaller, not the effort bigger)

0	1	2	3	4	5	6	7	8	9	10

What could get in the way — and what's my plan for it?

Diaphragmatic breathing — the 3-minute table-side script

"Put one hand on your chest, one on your belly. Breathe so the bottom hand moves and the top one barely does. In through the nose… two, three, four. Now out, slow, like fogging a mirror… two, three, four, five, six. The long, slow exhale is the lever — it's what tells your nervous system the emergency is over. Again…"

Run 5–6 cycles during care. When you feel the paraspinals let go, narrate it: *"Feel that softening? You did that. That's a skill, and it gets stronger every time you practice."* Prescribe like exercise: once a day, five minutes, logged, reviewed next visit.

Box breathing — for the patient who likes structure

"Four counts in · hold four · four out · hold four. Picture drawing the four sides of a box. Four boxes make a round; three rounds make a session." Good for at-work use — invisible and silent.

Progressive muscle relaxation — the short form

"We'll visit each muscle group: squeeze it tight for five seconds — tight enough to feel it, not enough to hurt — then let go all at once and notice the difference for ten. Feet… calves… thighs… glutes… belly… hands… arms… shoulders… jaw and face."

Teach the contrast: most patients with chronic pain live at 60% tension and call it zero. PMR recalibrates the gauge. Caution: skip or soften any region that's acutely flared.

Guided imagery — the outline

Patient picks one peaceful place (the only rule: it's calm). Guide through all five senses, slowly: what they see · hear · feel on the skin · smell · even taste. Two minutes in-office to seed it; the apps (p. 8) carry the home practice.

Objection handling, from the VA CBT-CP playbook: "I'm in too much pain to relax" → that's the stress–pain cycle talking; benefits are incremental. "I relax all the time — that's the problem!" → lounging isn't the same skill; this is active training. "I've tried, I can't" → the people who struggle most need it most; it's practice, not talent.

Calm the alarm — daily practice + better sleep

PATIENT HANDOUT

Long-term pain keeps your body's alarm system switched on — tense muscles, shallow breathing, poor sleep — and that tension turns the pain volume *up*. These skills turn it down. They work like exercise: a little, daily, beats a lot, rarely.

Your daily 5 minutes

- **Belly breathing:** hand on belly, slow breath in (4 counts), slower breath out (6). Ten breaths.
- **Box breathing:** in 4 · hold 4 · out 4 · hold 4. Three rounds.
- **Muscle release:** squeeze each muscle area 5 seconds, release 10 — feet to jaw.
- **Peaceful place:** close your eyes somewhere calm you love; visit it with all five senses.

Pick the ONE you like best — the one you'll actually do.

Sleep: the four that matter

- **Same wake time** every day — weekends too. It anchors everything.
- **Bed = sleep.** Can't sleep after ~20 minutes? Get up, do something dull in dim light, come back sleepy.
- **No clock-watching.** Turn it to the wall.
- **Caffeine curfew** ~8 hours before bed; wind down the last hour (screens dim, lights low).

Poor sleep turns up next-day pain — and pain wrecks sleep. Break the loop from the sleep side.

My practice log (check each day you practiced; jot how you felt after)

	Mon	Tue	Wed	Thu	Fri	Sat	Sun
Week 1							
Week 2							
Notes							

The boom–bust trap

Good day → do everything → crash for three days → fear grows → do less. Each loop shrinks what you can do.

Pacing: activity by the clock

Set a time dose you can manage *even on a bad day*. Stop when the timer says — **even if you feel great**. Raise it a small notch each week. Streak first; distance later.

My walking plan

Starting dose: _____ minutes Add each week: _____ minutes Review date: _____

Minutes walked	Mon	Tue	Wed	Thu	Fri	Sat	Sun
Week 1							
Week 2							
Week 3							
Week 4							

Re-expanding your life

Pain pushes the good things out of a life — and a life with nothing good in it hurts more. Pick three things you've missed. Schedule a small, paced version of each **on the calendar** — not “when I feel better.” Doing comes first; feeling better follows.

Something I miss	Small version of it	Day + time

Hurt doesn't always mean harm

Fresh pain is a working smoke alarm — it means check for fire. After months or years, the alarm's wiring becomes extra-sensitive: it can ring loudly for burnt toast. **The ringing is real.** The house is not burning down. Movement, practiced gradually, is how the wiring retrains.

The approach we're using has a name: CBT

Parts of your care plan come from **cognitive behavioral therapy (CBT)** — one of the best-studied tools in existence for people who want change. Change can even mean changing the cycle of chronic pain. It's not a secret technique, and you don't need a prescription for it: the resources below let you keep building on your own.

Free apps worth trying

- **Mindfulness Coach** (US Dept. of Veterans Affairs) — guided relaxation and mindfulness; free, no account
- **CBT-i Coach** (VA) — for pain-disrupted sleep
- **Insight Timer** — large free library of guided sessions; search “pain,” “body scan,” or “sleep”

Free reading & workbooks

- **VA Cognitive Behavioral Therapy for Chronic Pain** patient materials — breathing, relaxation, pacing, and activity worksheets, free online
- Search terms that find good material: *“CBT for chronic pain workbook,” “pain neuroscience education”*
- Ask us for a referral to a CBT-trained therapist any time — it's a normal part of pain care, not a last resort

The one-sentence summary of everything: your pain is real; your nervous system is trainable; and the person with the most power over this condition is you. We're your coaching staff.

More materials from this program — including the course slides your clinician learned this from — at **ChiroSmarts.com/talking-to-pain**